

Tirzepatide

Patient Education Handout

Weight Loss / GLP-1 Program · KORB Health Group · For Patient Reference

This handout is for educational reference only. It is not medical advice. Always follow your prescription label, your pharmacy-specific instructions, and your KORB provider's guidance. If you have questions, contact KORB by phone or email (see contact information below).

What Is Tirzepatide

Tirzepatide is a dual GIP (glucose-dependent insulinotropic polypeptide) and GLP-1 receptor agonist. It works through two separate gut hormone pathways rather than one, which slows digestion and reduces appetite — often more strongly than a single-pathway medication, for some patients.

Tirzepatide is FDA-approved for weight management and for type 2 diabetes. At KORB, your provider prescribes a compounded formulation that includes a small amount of cyanocobalamin (vitamin B-12) to help with tolerability.

Possible Support From Tirzepatide

- Weight loss — often more pronounced than single-pathway GLP-1 medications, for some patients
- Increased energy as excess weight decreases
- Better sleep quality for some patients
- Improved cardiovascular health markers
- Support for blood sugar management, if applicable to you

Response to Tirzepatide varies from patient to patient. Most patients notice reduced appetite within the first few weeks, with weight loss building gradually over months. **Tirzepatide is a tool that supports weight loss — it is not a “magic shot” that works on its own.** It works best paired with the nutrition and activity habits below, not as a substitute for them.

Nutrition & Lifestyle Tips That Support Your Results

These habits help your body respond well to Tirzepatide and reduce the chance of GI side effects. Ask your provider if you'd like more individualized nutrition guidance.

- Prioritize protein at each meal — it helps preserve muscle mass while you lose weight.
- Eat slowly and stop when you feel satisfied, not full — Tirzepatide slows digestion, so it takes longer to feel full and easy to overeat before your body catches up.
- Eat smaller, more frequent meals if large meals feel uncomfortable.
- Avoid greasy, fried, or very high-fat foods — these commonly worsen nausea on GLP-1/GIP medication.

- Stay well hydrated throughout the day.
- Limit alcohol, which can worsen nausea and interferes with steady progress.
- Stay physically active as you're able — movement supports muscle retention and overall results.
- Prioritize consistent sleep — it supports appetite regulation and overall metabolic health.

How to Use Tirzepatide

Your prescription label — or the instructions given to you by your KORB provider — is the authoritative source for your dose and directions. What follows is a general reference. Do not adjust anything without speaking to your provider first.

How to inject	When to inject	Schedule	Titration
Subcutaneous (SQ) injection — fatty tissue under the skin (abdomen, thigh, or back of arm)	Same day each week, with or without food	Once weekly	Dose increases every 4 weeks, only as directed by your provider

Your Titration Schedule

Your provider starts you at a low dose and increases it gradually — typically every 4 weeks, based on how you're tolerating your current dose. The exact milligram steps and injection volume depend on which pharmacy fills your prescription, since KORB works with more than one compounding pharmacy and each uses a slightly different concentration and step schedule.

Your prescription label always reflects your correct dose and volume for your specific pharmacy. For a quick reference showing your injection volume and syringe markings by pharmacy and dose, ask your provider about KORB's GLP-1 Dose & Injection Guide tool.

Do not increase your own dose, even if you feel ready or your symptoms are mild. Increasing too quickly raises the risk of nausea and other side effects. If a dose isn't well tolerated, tell your provider — they may extend that step before increasing further.

Missed a Dose?

- If it's been less than 4 days since your missed dose, give the injection as soon as you remember, then resume your normal weekly schedule.
- If it's been 4 days or more, skip the missed dose and take your next dose on your regularly scheduled day. Do not double up.

Traveling with Your Medication

Keep your medication with you — in your carry-on or personal item, not checked baggage — in its original labeled vial or packaging. It doesn't need to be packed in ice during travel; just keep it away from excessive heat (above 86°F / 30°C) and from freezing. Once you reach your destination, refrigerate it again. Medication and syringes are permitted in carry-on luggage; TSA recommends keeping the original pharmacy label visible.

Storage and Handling

Refrigerate	Do not freeze	Protect from light	Discard at 28 days
36°F – 46°F (2°C – 8°C)	Freezing damages medication	Keep away from direct sunlight	Write open date on vial

Before first use: refrigerate, protect from light, do not freeze or shake. After opening: write the open date on your vial and discard 28 days after first use — even if medication remains. Do not use if the solution appears cloudy, discolored, or contains particles.

What to Expect — Realistic Timeline

Timeline	Phase	What to Expect
Weeks 1–4	Starting out	Reduced appetite often begins here. Mild nausea is common as your body adjusts.
Weeks 4–20	Titration	Dose increases every 4 weeks as tolerated. Weight loss builds gradually.
Months 5–8	Continued progress	Many patients reach a meaningful maintenance dose in this window.
Ongoing	Maintenance	Most patients remain on a steady dose long-term to sustain results.

Do not compare your response to others. Response is highly individual and depends on starting weight, metabolism, diet, activity, and other factors.

Side Effects and What to Watch For

What You May Notice — Common	What to Do
Nausea, vomiting, or diarrhea	Common, especially after a dose increase. Eating smaller, lower-fat meals can help. Tell your provider if severe or persistent.
Constipation or stomach pain	Common. Stay hydrated and tell your provider if it doesn't improve.
Low appetite	Expected and part of how the medication works. Tell your provider if you're unable to eat or drink adequately.
Injection site redness or irritation	Common, usually mild. Rotate injection sites.
Headache or dizziness	May occur early in therapy. Usually mild and short-lived.

Worth Monitoring — Tell KORB Provider

- Severe or persistent abdominal pain, especially if it spreads to your back — can indicate pancreatitis.
- Signs of gallbladder problems: pain in the upper right abdomen, fever, or yellowing of the skin or eyes.
- Inability to keep down fluids for more than 24 hours.

Stop injecting and seek emergency care immediately if you experience: trouble breathing, swelling of the face, lips, tongue, or throat, severe rash or hives, chest pain, or any symptom that feels severe or unsafe.

Who Should Not Use Tirzepatide

- Personal or family history of Medullary Thyroid Carcinoma (MTC) or Multiple Endocrine Neoplasia syndrome type 2 (MEN2)
- Known hypersensitivity to Tirzepatide or any component of the formulation
- Current pregnancy, breastfeeding, or planning pregnancy
- Active gallbladder disease or history of gallbladder-related surgical complications
- History of pancreatitis (use with caution — discuss with your provider)
- Severe gastrointestinal disorders (such as gastroparesis) or severe kidney impairment

Lab Monitoring

Routine labs are not required to participate in this program. If you are also managing diabetes or pre-diabetes, continue that care — including any related labs — with your primary care provider alongside your KORB visits. If your KORB provider has an individual clinical reason to order labs for you, they'll explain why and what to expect.

Safety Reminders

- Use Tirzepatide only as prescribed. Do not change your dose, schedule, or route without provider direction.
- Do not combine with other GLP-1 or GIP medications from another source at the same time.
- Tirzepatide is not appropriate during pregnancy, breastfeeding, or with a personal/family history of MTC or MEN2.
- Tell KORB provider if you develop any new medical condition, start a new medication, or become pregnant.
- Your prescription label is the authoritative source for your dose and instructions.
- Use a new insulin needle with syringe for every injection — do not reuse.
- Follow your state or local regulations for sharps disposal — use an FDA-cleared sharps container.

When to Contact KORB and When to Seek Emergency Care

Contact KORB Operations	Patient Portal — Message Your Provider	Seek Emergency Care Immediately
Questions about timing, storage, or administration Email: info@korbhealth.com Phone: +1 (888) 959-7299 Hours: Mon–Fri, 9 AM–6 PM CT	- Mild but persistent side effects - Questions about whether to continue therapy - Questions about dose increases, tolerability, or nausea management	- Trouble breathing or throat / face swelling - Severe rash or widespread hives - Chest pain or fainting - Severe or rapidly worsening symptoms - Confusion or severe weakness - Any symptom that feels urgent or unsafe

The Patient Portal is your only HIPAA-compliant way to message your provider directly — providers check portal messages once per day on clinic days. Phone and email are not appropriate for emergencies. When in doubt, go to urgent care or the emergency room.

Key Reminders

- Tirzepatide works through two hormone pathways and may support more pronounced weight loss for some patients.
- Dose increases happen every 4 weeks and only under provider direction — never increase on your own.
- Write the open date on your vial — discard 28 days after first use.
- Nausea is common, especially after a dose increase — tell your provider if it's severe or persistent.
- Routine labs are not required for this program.
- Contact KORB Operations by phone or email for non-urgent questions, or your provider via the Patient Portal for medical questions. Seek emergency care for severe symptoms.